

Policy on the Medical Record Reflecting Continuity of Care

«Hospital Name»

Document control

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Prepared by	Medical Records In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

Statement of intent

«Hospital Name» ensures the medical record reflects the continuity of care a patient receives, from admission through assessments, procedures, transfer or discharge, and remains accessible to care providers.

1. Purpose

This policy sets out how the medical record at «Hospital Name» documents reasons for admission, diagnosis and care plan; assessments, re-assessments and consultations; investigation results and care provided; operative and other procedures; transfer details; the discharge summary; the cause of death where applicable; and access by care providers to current and past records.

This policy does not cover the medical record's unique identifier, entry authorisation, or confidentiality and security of records — those are covered in other hospital policies.

2. Scope

This policy applies to the content of the medical record for every patient cared for at «Hospital Name», across admission, in-patient stay, transfer, discharge and death where applicable.

3. Policy standards

The medical record at «Hospital Name» contains information on reasons for admission, diagnosis and care plan.

The medical record contains details of assessments, re-assessments and consultations.

The medical record contains investigation results and details of care provided.

Operative and other procedures performed are incorporated in the medical record.

When a patient is transferred to another organisation, the medical record contains the transfer details.

The medical record contains a signed copy of the discharge summary.

In case of death, the medical record contains a copy of the medical certificate of the cause of death.

Care providers have access to the current and past medical record.

4. Non-negotiable rules

1. Do not admit an in-patient without the treating doctor or a doctor member of the treating team documenting the diagnosis, preferably coded per ICD/SNOMED CT.

2. Do not leave assessments, re-assessments or consultations — medical, nursing, rehabilitation, physiotherapy, nutrition — out of the medical record.
3. Do not leave investigation results or details of the care provided out of the medical record.
4. Do not perform an operative or other procedure without incorporating its name and details into the medical record.
5. Do not transfer a patient to another organisation without recording the date, reason, receiving organisation, and the patient's clinical condition before transfer.
6. Do not discharge a patient without a discharge summary signed by a doctor member of the treating team.
7. Do not omit the medical certificate of cause of death, coded per the WHO International Classification, when a patient dies — and never record cardiac or respiratory arrest as the cause of death.
8. Do not deny a designated care provider access, including 24-hour access, to a patient's current and past medical record needed for continuity of care.
9. Staff who see a rule under this policy broken report it the same shift to the Medical Records In-Charge or the Medical Superintendent.

5. What we do

5.1 Reasons for admission, diagnosis, care plan

For in-patients at «Hospital Name», the diagnosis is documented by the treating doctor or a doctor member of the treating team in all records, preferably per ICD/SNOMED CT. In the medical records department, all diagnoses are codified per ICD/SNOMED CT.

5.2 Assessments, re-assessments and consultations

Details of assessments, re-assessments and consultations at «Hospital Name» — medical, nursing, rehabilitation, physiotherapy, nutrition — form part of the medical record, in physical or electronic form.

5.3 Investigation results and care provided

Results of all investigations at «Hospital Name» form part of the medical record, in physical or electronic form, consistent with how the medical record's contents are defined in other hospital policies.

5.4 Operative and other procedures

The name and details of operative and other procedures performed at «Hospital Name» are incorporated in the medical record.

5.5 Transfer details

When a patient is transferred to another organisation, the medical record at «Hospital Name» contains the date of transfer, the reason for transfer, and the name of the receiving organisation. The patient's clinical condition before transfer is mandatorily recorded, consistent with how transfer summaries are handled in other hospital policies. If the patient requested the transfer, a note to that effect may be added, and the receiving organisation recorded is the one the patient desires to go to.

5.6 Discharge summary

The medical record at «Hospital Name» contains a signed copy of the discharge summary, signed by a doctor member of the treating team. Electronically signed discharge summaries are acceptable.

5.7 Cause of death

In case of death, the medical record at «Hospital Name» contains a copy of the medical certificate of the cause of death, stating the cause, date and time of death, coded per the WHO International Classification of the Cause of Death. Cardiac and respiratory arrest is recorded as an event of death, not the cause of death. The certificate is completed following the guidance in the "Physicians' Manual on Medical Certification of Cause of Death" issued by the Office of the Registrar General, Ministry of Home Affairs, Government of India.

5.8 Care provider access

«Hospital Name» provides access to medical records to designated care providers involved in a patient's care. For electronic medical record systems, identified care providers hold a user ID and password. A 24-hour provision ensures the patient's record is available to healthcare providers for continuity of care. For physical records, when the medical records department is closed, a system allows authorised personnel to access and retrieve the record; records are retrievable for all existing hospital patients presenting to the emergency room.

6. Governance and responsibility

Role	Responsibility
Medical Records In-Charge	Owens this policy. Maintains the 24-hour access mechanism and transfer/discharge/death record completeness checks.
Treating Doctor / Care Team	Documents diagnosis, assessments, procedures, transfer details, discharge summary and cause of death.
Medical Superintendent	Oversees implementation of this policy. Receives escalations under Section 4.
All Staff	Follow access-authorisation procedures for medical records. Report identified risks and rule breaches.

7. Quality monitoring

The Medical Records In-Charge reviews sampled records for admission/diagnosis documentation, assessment completeness, transfer and discharge summary presence, cause-of-death certification accuracy, and 24-hour care-provider access functioning. This policy is reviewed annually, or sooner if NABH standards change.

8. Training and staff acknowledgement

All staff who document in or access the medical record are trained on continuity-of-care documentation requirements, and sign the acknowledgement below.

Acknowledgement statement:

"I have read the Policy on the Medical Record Reflecting Continuity of Care of «Hospital Name». I will follow the processes described."

Signature table (one row per staff member):

Staff Name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy is distributed to the Medical Records department, all clinical department heads, and is made available to all staff through the hospital intranet and department policy folders.

10. Abbreviations

Abbreviation	Meaning
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
IMS	Information Management System (NABH Hospitals 6th Edition chapter)

OE	Objective Element
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11. Traceability to NABH HCO Full Accreditation 6th Edition — IMS.4

This table is an index. It is not how the process is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
IMS.4.a	Commitment	Admission reasons, diagnosis, care plan documented	Section 5.1	Treating Doctor / Care Team
IMS.4.b	Commitment	Assessments, re-assessments, consultations documented	Section 5.2	Treating Doctor / Care Team
IMS.4.c	Commitment	Investigation results and care provided documented	Section 5.3	Treating Doctor / Care Team
IMS.4.d	Commitment	Operative/other procedures incorporated	Section 5.4	Treating Doctor / Care Team
IMS.4.e	Commitment	Transfer details documented	Section 5.5	Medical Records In-Charge
IMS.4.f	Commitment	Signed discharge summary in record	Section 5.6	Treating Doctor / Care Team
IMS.4.g	Commitment	Medical certificate of cause of death in record	Section 5.7	Treating Doctor / Care Team
IMS.4.h	Commitment	Care providers have access to current/past record	Section 5.8	Medical Records In-Charge

12. Required Records / Evidence Checklist

IMS.4.a — Admission, diagnosis, care plan.

- Documented diagnosis record by the treating doctor/team, coded per ICD/SNOMED CT in the medical records department.

IMS.4.b — Assessments/consultations.

- Medical, nursing, rehabilitation, physiotherapy and nutrition assessment records in the file.

IMS.4.c — Investigations and care.

- Investigation-result records and care-provided documentation in physical/electronic form.

IMS.4.d — Operative/other procedures.

- Procedure-name and detail record incorporated in the medical record.

IMS.4.e — Transfer details.

- Transfer record — date, reason, receiving organisation, pre-transfer clinical condition — and patient-requested-transfer note where applicable.

IMS.4.f — Discharge summary.

- Signed (or electronically signed) discharge summary record in the file.

IMS.4.g — Cause of death.

- Medical certificate of cause of death, WHO-classification-coded, following the Registrar General's Physicians' Manual.

IMS.4.h — Care provider access.

- User ID/password access record for electronic systems, and after-hours physical-record access/retrieval record.
- Emergency-room record-retrieval confirmation for existing hospital patients.

13. References

- NABH Guidebook for Accreditation of Hospitals, 6th Edition — Standard IMS.4, "The medical record reflects the continuity of care."
- World Health Organization. (2022). International Classification of Diseases, 11th Revision (ICD-11). <https://www.who.int/standards/classifications/classification-of-diseases> — the classification system named in IMS.4's own text for coding cause of death.
- Office of the Registrar General, Ministry of Home Affairs, Government of India. (2012). Physicians' Manual on Medical Certification of Cause of Death. https://ncdirindia.org/e-mor/Download/Physician's_Manual_MCCD.pdf — the manual named in IMS.4's own text for completing the cause-of-death certificate.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

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